



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : مشيره احمد حسين احمد
المحافظة : الجيزه الحي / المدينة : امبابه
رقم البطاقة : 28506292104029 رقم التليفون : 01221132022 تاريخ تسجيل الإستمارة : 29-09-2026
العمر(تاريخ الميلاد) : 29-06-1985 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
المقر بما فيه
التوقيع: (-----) (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: مشيره احمد حسين احمد National ID: 28506292104029	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

مشير

احمد حسين احمد

٨ ش صلاح منال - ش المطار - المليحة غ

امبابه - الجيزة

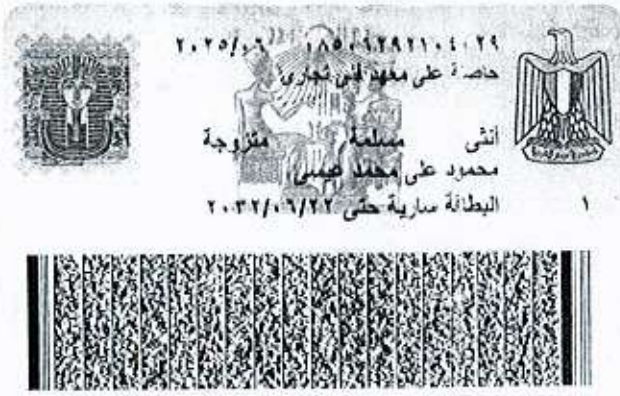
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DMS

مقاطعة مريوط
بالهجرة
ماموجرام



7/28/26, 4:51 PM

code:09 - Name_A MDT FORM -- Name_E MDT FORM

		جمهورية مصر العربية لدعم صحة المرأة المصرية		
07/28/2026				
MDT FORM				
Name:	محمود احمد حسن احمد	MRN:	11886	
Age:	41 Y	ID:	26506292104029	
Menopausal Sta	PRE	Phone:*	01106116073	
NRS SCORE:	PS 0	Marital Status:	MARRIED	
BMI:	20	Family History:	-VE	
Gender		Reason Of MDT:	NEW CASE	
CO Morbidity:	NAD	ECOG Performance Status	☒ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4	
Surgical History:*	CS			

Clinical Data:

BX: IDC G3
 ER: 0/8 PR: 0/8 HER2: 0+ ULTR K167: 80

History:
 DELIVERED FILMS TO MDT FOR REVISION

Sonomammography:
 ACR B
 RT BREAST UIQ 12 O'CLOCK 42/37/38 MM SCH FROM NIPPLE, ABUTTING
 OVERLING SKIN
 ANOTHER 9-10 O'CLOCK 15X5.6MM
 MULTIPLE PATHOLOGICAL ENLARGED LEVEL I, II
 LT INDETERMINATE LN (BIOPSIED - VE)
Biomarker/Molecular

Mets Workup:
 CT CAP FREE (RT SMALL SCLEROTIC LESION FOR BONE SCAN)
 ECHO FREE
 BONE SCAN : free
Recommendations:
 FOR NACT AND CLIPPING OG BOTH LESIONS THEN RT CBS AND ALND

MDT Outcome & Plan
 CLIPPING OF BOTH LESIONS
 Portacath Insertion
 NACT: Keytruda + Taxol - Carbo x 12 week --> AC + Keytruda x 4 cycles
 THEN RT CBS AND ALND

Clinical Staging:Pathological Staging:SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. علاء عبد العزيز	د. محمد كبري		

Pat. Name: Moshira Ahmed Hussein	Pat. ID: 14471896
Procedure Name: bone Scintigraphy	

Nuclear Medicine Unit

Patient Name: MOSHIRA AHMED HUSSIEN

Date of Scan: 26/07/2026

ISOTOPIC BONE SCAN REPORT

Clinical History

41-Y/O lady; recently diagnosed as right breast cancer; referred for metastatic work up.

TECHNIQUE

Bone scintigraphy done 2 hrs. following I.V injection of 20 mCi of Tc-99m M.D.P.

FINDINGS

- The scanned skeleton keeps free from any definite scintigraphic evidence of abnormal active or cold focal bony lesions that may account for metastatic bone disease.
- Degenerative changes are noticed at the spine and big joints.
- Normal visualization of both kidneys.

IMPRESSION

The scanned skeleton is currently scintigraphically free from metastases.

Thanks for reference,

Dr/Ahmed NASR, M.D.
Consultant of Nuclear Medicine
(AN)

Patient Name	مشيره احمد حسين	Case No.	328776
Sex/Age	Female 41 Y	Patient No.	24183
Registered Date	27/6/2026 11:11:55 AM	Referred By Dr.	Out Patient
Validation Date	13/7/2026 03:03:01 PM	Branch	Gustave Roussy
Reporting Date	25/7/2026 11:25:20 AM	Pathology No.	SR-753/26

Breast Slide Revision

Clinical Data: Right breast mass (BIRADs 5).

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (1066512). Together with, five referred slides including 1 (H&E) stained slide and 4 immunostained slides for (ER, PR, Her2 & Ki67) labelled by (780069).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Necrosis: Present.

IPT:

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Faint in complete membrane staining in less than 10% of tumor cells (score 0+).
- Ki67%: 80%.

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.

IPT:



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	مشيره احمد حسين	Case No.	328776
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- ER: Negative (0/8).
- PR: Negative (0/8).
- Her2: Ultra-low (Score 0+).
- Ki67%: 80%.

* *Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* **Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

PATIENT NAME: MOSHERA AHMED HUSEN AHMED

GENDER: Female

DATE : 07-Jul-2026 10:58 AM

PATIENT ID: 20226031566

AGE: 41 y

ORDER: Z455689,Z455690

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast pathologically proven malignant mass lesion (IDC grade III).

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
- **RIGHT BREAST:**
- Right breast UIQ 12 O'clock posterior third an irregular, speculated dense mass lesion, it measured about 42x37x38mm, it is about 8cm from the nipple.
- Complementary ultrasound showed right 12 O'clock zone c irregular, speculated, hypoechoic solid mass lesion, **seen abutting the overlying skin**, it measured about 31x30.5x40mm.
- Right 9-10 O'clock a lobulated, parallel, circumscribed lesion of mixed echogenicity, it measured about 15x5.6mm, it is about 6.6cm from the nipple & 6.2cm from the malignant mass lesion.
- Multiple pathologically enlarged level I-II & infraclavicular lymph nodes, the largest seen at level I measured about 19x20mm.
- Free supraclavicular region.
- **LEFT BREAST:**
- No speculated mass lesions or grouped microcalcifications.
- Complementary Ultrasound showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Few enlarged level I axillary lymph nodes with preserved central fatty hila & diffuse cortical thickness reaching up to 5mm.
- Free level II, infra & supraclavicular regions.

OPINION:

PATIENT: MOSHERA AHMED HUSEN AHMED, 20226031566, Female, 41 y

Report version 1, 07-Jul-2026 12:17 PM

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16602

مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤٠ الجي الأول - الشيخ زايد أمام كلية الهندسة

- Right breast UIQ pathologically proven malignant mass lesion & ipsilateral pathologically enlarged level I-II & infraclavicular lymph nodes (BIRADS6).
- Left axillary indeterminate lymph nodes for FNAC.
- Left breast no suspicious findings(BIRADS2).

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Noora, El Kashef,FEBR.EBWI

07-Jul-2026 12:17 PM

PATIENT: MOSHERA AHMED HUSEN AHMED, 20226031566, Female, 41 y

Report version 1, 07-Jul-2026 12:17 PM

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16602

مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤٠ العي الأول - الشيخ زايد أمام كلية الهندسة

Mammogram

[[DOCTYPE:MAMMOGRAM]]

Name:	Moshirah Ahmed Hussein Ahmed	Age :	40
Referred by:	Prof. Dr/ Amr Salah	Date :	07/06/2026
Examination:	MAMMOGRAPHY		

BILATERAL SONO-MAMMOGRAPHY:

- The right breast showed hyper dense lesion in upper quadrant with marginal speculations and by ultra sound the mass is measuring about (2.3x2.1cm with 2.5 cm depth) at 12 o'clock and showed posterior acoustic shadowing with noted rounded right axillary lymph nodes with fatty hilum loss.
- The left breast is showing normal size, shape, nipple, areola, skin and subcutaneous tissue thickness.
- The left breast is showing normal distribution of the glandular, fatty and vascular tissue elements, with predominant fatty elements with no breast masses seen & no malignant micro calcification detected with no axillary lymphadenopathy and the complimentary sonography revealed absence of any solid or cystic breast masses.

CONCLUSION

- Right breast suspicious mass (BIRADS 5).

Best regards,
 Dr. Mohamed A. Fouad, FEBR
 Radiology consultant

BIRADS

1	normal	
2	benign finding	
3	probably benign finding	6-month follow-up
4	suspicious finding	biopsy recommended
5	high likelihood of malignancy	biopsy recommended
6	confirmed malignancy	

Patient Name: MOSHERA AHMED HUSEN AHMED	Gender: Female
MRN: 20226031566	Ordered By: Elaria Toba Tawfik Yacoub
DOB: 29/06/1985 Age: 41 Y 0 M 24 D	Order ID: 455899

CT CHEST w contrast

Procedure Date: 08/07/2026 10:06:00 AM

Report:

Report:
POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH
TRIPHASIC STUDY OF THE LIVER

Technique:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

FINDINGS:

1-CHEST

- Right breast UIQ irregular speculated enhancing mass lesion with central breakdown, measuring about 30 x 31 mm along axial dimensions.
- Multiple pathologically enlarged right axillary level I and II lymph nodes, the largest is level I measuring about 18 x 22 mm with central breakdown.
- Few enlarged left axillary level I and II lymph nodes, the largest is level I measuring about 9 x 12 mm.
- Bilateral few infraclavicular lymph nodes, the largest seen at the right side measuring about 6 x 10 mm.
- Bilateral few internal mammary lymph nodes, the largest is on the left side measuring about 3 x 7 mm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Middle lobe fine atelectatic band.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- Mildly enlarged liver with homogenous CT texture showing segment IV b hypodense area with no appreciable post-contrast enhancement measuring about 7 x 12 mm (likely focal fat infiltration). No definite focal lesions, no intra-hepatic or extra-hepatic biliary radicles dilatation..
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I.V.C and the pelvic organs.
- Underfilled urinary bladder.
- No pathologically enlarged abdominal or pelvic lymph nodes.

Patient Name: MOSHERA AHMED HUSEN AHMED	Gender: Female
MRN: 20226031566	Ordered By: Elaria Toba Tawfik Yacoub
DOB: 29/06/1985 Age: 41 Y 0 M 24 D	Order ID: 455690

US. Both breast

Procedure Date: 07/07/2026 10:45:00 AM

Report:

Report:

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast pathologically proven malignant mass lesion (IDC grade III).

TECHNIQUE:

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- Complementary ultrasound

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- Scattered fibroglandular densities (ACR B).
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- Free supraclavicular region.
- Left breast:
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- Free level II, infra \/\ supraclavicular regions.

OPINION:

- Right breast UIQ pathologically proven malignant mass lesion \/\ ipsilateral pathologically enlarged level I-II \/\ infraclavicular lymph nodes (BIRADS6).
- Left axillary indeterminate lymph nodes for FNAC.
- Left breast no suspicious findings(BIRADS2).

Approved by: Noora, El Kasher, FEBR.EBWI